NCCP 00789 · clinician draft

# TCHP with Phesgo

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Postoperative HER2-only phase · confirm selected regimen

**Active medicines:** Selected postoperative HER2 medicine; separate source regimen

**Emetogenic risk:** Minimal for Phesgo / trastuzumab alone

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

### Additional phase-specific support

- No routine prophylactic antiemetic or cytotoxic premedication is carried over. Previous reactions or symptoms need an individual plan.
- Phesgo premedication is not usually required; consider paracetamol/antihistamine for the individual reaction history.
- NCCP 00789 describes postoperative trastuzumab for a further 12 cycles to complete about one year from the first dose. Pathology, including residual invasive disease, can change the adjuvant medicine; confirm a separate approved regimen. Do not route early-breast patients automatically to metastatic Phesgo maintenance 00785.
- Continue cardiac monitoring; maintenance does not restart the echo clock.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00789 v3 · p8 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-20 · SACTCheck v0.84.12 · Regimen-level drafts; prescriber and oncology-pharmacy verification required